

Unusual Neurological Diseases, Somatoform Disorders and Factitious Disorders

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Session Objectives:

- 1) Distinguish neurological disorders from psychiatric disorders.
- 2) Recognize when psychiatric disease is comorbid with neurological disorders.
- 3) Identify when a patient is causing harm to him/herself.
- 4) Implement proper treatment for these conditions.

Case 1

- A 60-year-old woman is struggling to get dressed. While the right hand is buttoning, the left hand is unbuttoning.
- Is this a neurologic or psychiatric condition?
- What is this called?
- What condition would you expect to see it in?

Alien Limb Phenomenon

- Seen in 30-50% of patients with corticobasal degeneration.
- May affect arm or leg.
- Moves outside of voluntary control. Able to perform a complex action.
- Limb has a “mind of its own.”
- The opposing hand may hold down the “alien limb”.

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Case 2

- This is a 56-year-old man brought to the ED by family with strange behavior.
- On examination- difficulty following your finger, and bombed the visual fields.
- He compliments you on glasses you're not wearing.
- On the gait exam he walks into the storage closet.

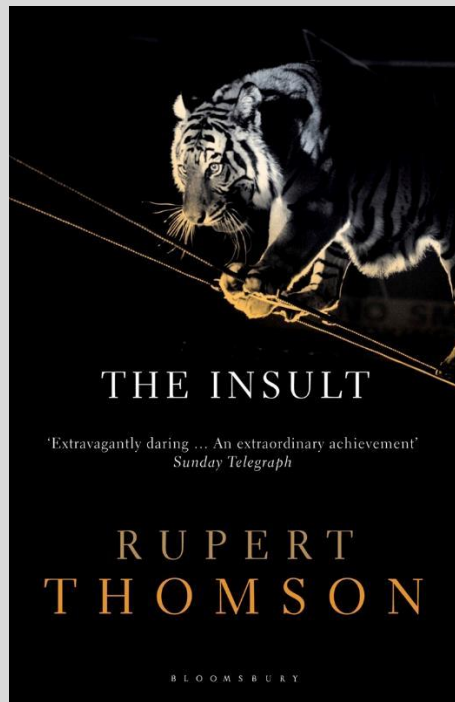
Anton syndrome

- AKA Anton-Babinski syndrome is visual anosognosia (denial of vision loss), associated with confabulation to mask the deficit.
- Etiology is damage to bilateral occipital lobes producing cortical blindness.
- Lack of awareness of their deficit
- Very rare condition

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Case 3

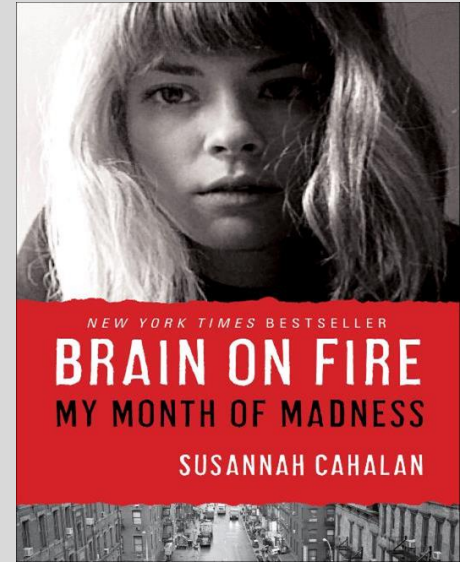
- A 17-year-old girl presented with recent difficulty in school.
- A few months ago she had a flu like illness.
- First memory became a problem, then she became agitated and convinced that the neighbors were watching her.
- During the exam her legs began moving in a pedaling motion...this was followed by a generalized seizure.

NMDAR encephalitis

- An autoimmune encephalitis that may be associated with ovarian teratoma or other rare tumor.
- Patients initially present with neuropsychiatric symptoms.
- Seizures may occur that are resistant to antiepileptic drugs (AEDs).
- May present with unusual movement disorders.

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Case 4

- A 12-year old girl is brought to clinic because of difficulty walking (it's a morning appointment)
- The parents claim that by the evening she has to be carried.
- Her parents are perplexed because she walks fine in the morning. She is not having difficulty walking at this time.

Dopa responsive Dystonia

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- Also DYT 5 or Segawa disease
- Often presents with gait abnormality due to lower extremity dystonia. May be mistaken for cerebral palsy or spastic diplegia.
- Diurnal fluctuation: symptoms worsen in afternoon and improve in the morning after sleep.
- Treatment: Remarkable response to low dose levodopa.

Case 5

- A 20-year-old man reports that whenever he stands up quickly the muscles on the left side of his body become extremely tight.
- It often causes him to fall.
- He has difficulty reproducing this in the office.

Paroxysmal Kinesigenic Dyskinesia

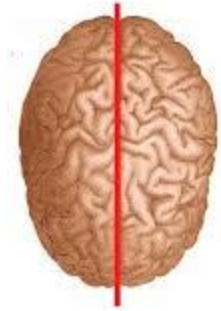
- Dystonic posture or dyskinesia is brief- lasts seconds to minutes.
- Attacks triggered by sudden voluntary movements, stress, startle or sleep deprivation. May occur 100 times/day in some cases.
- Seizures may also be present.
- Responds to carbamazepine or other anti-seizure medications.

Case 6

- A patient is blindfolded and given an object to hold in his left hand. He cannot name the familiar object.
- When it is transferred to the right hand, he is able to name the object.

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Split brain syndrome

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- Callosal disconnection syndrome
- Although rare, this is the result of a patient who has had a lesion of the corpus callosum. May be due to:
- Surgery, MS, agenesis of corpus callosum
- Stroke, tumor, Marchiafava-Bignami disease
- Left hemisphere- calculations, reading and speech.
- Right hemisphere- spatial tasks

Case 7

- A 60-year-old man is brought to the ED acutely confused.
- His wife relates that they were out to dinner and suddenly he began asking “where are we? “what’s the date” over and over.
- He is able to repeat digit spans on exam.

Transient Global Amnesia

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- Acute onset of anterograde amnesia (inability to form new memories).
- Repeatedly asks the same question reflecting disorientation.
- Other cognitive functions are normal.
- Eventually return to normal (generally within 24 hours).
- Presumed to either be migraine, ischemic, epileptic or psychogenic in origin.

Transient
Global
Amnesia



Case 8

- A 25-year-old woman is admitted to the hospital with high fever and is found to have endocarditis.
- On further investigation it is determined that she works in a microbiology lab. She is found to be infected with a rare infectious agent.

Case 9

- A 27 year old woman comes in to the clinic in a wheelchair. She brings a chart the size of an encyclopedia, which is organized with dividers.
- She has already seen every doctor that you know, and has visited Mayo- twice.
- She suggests a muscle biopsy, because it is the one procedure she has not managed to have yet.
- Refuses to speak to a counselor.

Factitious Disorder

- Occurs when an individual falsifies physical or psychological signs or symptoms or induces injury or disease with intended deception.
- The individual may create signs or symptoms by 1) lying 2)simulating-putting drops of blood into urine sample 3)actually creating disease by injecting bacteria.

Factitious Disorder

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- Primary aim is to achieve the patient role.
- Behavior persists even in the absence of monetary rewards or drugs, etc.
- The individual may seek invasive diagnostic testing, surgery or treatment.

Munchausen's Syndrome

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- This is the most severe variant of factitious physical disorder.
- Exaggerated lying AKA: pseudologia fantastica, sociopathic behavior, geographic wandering from hospital to hospital.
- Lives a continuous life of “patienthood”.
- The patient may also use aliases.

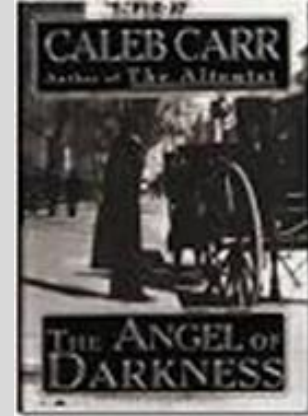
Case 10

- A 12-year-old girl has missed school again.
- She is in another doctor's office.
- She is extremely thin, tired and weak.
- It is 4 pm and her mother would not allow her to eat all day again.
- Mom is about to suggest open heart surgery to the doctor.

Munchausen by Proxy

- This is a form of child abuse and elder abuse that may easily be overlooked.
- The caregiver causes the sickness.

- Sickened
- Angel of Darkness
- More than it Hurts you



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Case 11

- A 22 year old woman is obsessed that her thighs are extremely fat and she comes to the PCP (you) for medical clearance before a major fat trimming surgery.
- On examination she has a normal build and her legs are proportionate, but not heavy.
- What condition do you suspect?

Body Dysmorphic Disorder

- BDD is characterized by a preoccupation with a perceived defect or flaw in the patient's appearance.
- Generally the flaw is not observed by others and the patient becomes obsessed with this particular flaw.
- The preoccupation triggers repetitive behaviors which are difficult to control and are not pleasurable.

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Case 12

- A 25-year-old woman from El Salvador presents to the Ob-Gyn clinic. She states that the reason for her visit is to establish care in order to deliver her baby.
- She appears to have a gravid abdomen, but after extensive testing, it is determined that she is actually not pregnant.

Pseudocyesis

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- The false belief that a patient is pregnant including growing abdomen, morning sickness and cessation of menstruation.
- Unclear if due to hormones or in some cases may be psychiatric.



Case 13

- A 26-year-old woman is brought to the hospital soon after the death of a close family member.
- A friend who brought her states that she suddenly became paralyzed.
- When you ask her to walk, she has an extremely bizarre gait and nearly falls, but does not.

Functional neurological Disorder

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- Neurologic symptoms such as weakness, abnormal movements, or nonepileptic seizures without structural disease.
- Clinical findings on exam are not compatible with recognized neurologic disease.
- The disorder causes distress and/or functional impairment and has a poor prognosis.

Case 14

- A 20-year-old woman presents to the ED after a minor MVA.
- She complains of numbness down the left side of her body.
- A vibrating tuning fork is placed on her forehead. She insists that she feels the vibrations on the right, but not on the left.

Malingering

- The patient consciously creates physical or psychological symptoms to assume the sick role with the goal of receiving a reward.
- The reward may include money, an insurance settlement, drugs, release from jail, avoidance from punishment, military, etc. May occur after a car accident (no fault).
- This is not a psychiatric illness and was not included in the DSM-V.

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Case 15

- A 35-year-old woman reports feeling “pins and needles” in her hands and feet.
- She calls me concerned that she has a brainstem tumor (she googled her symptoms).
- If not a brainstem tumor, then it's MS.
- She is preoccupied with her condition and it affects her life.

Illness Anxiety Disorder

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- Excessive concern about having or developing a serious undiagnosed general medical disease. Previously patients were diagnosed with hypochondriasis in the DSM-4-TR.
- Patient has a belief that they have a specific disease. In DSM-5-TR



Case 16

- A patient is sitting in the clinic exam room. Suddenly she stares off into the distance. She then begins to furiously kiss the cross that hangs from her neck.
- This goes on for 2 minutes. During this time she does not respond to examiner's questions.
- Slowly she returns back to her baseline.

Focal seizure with dyscognitive features

- Seizures involve abnormal electrical impulses within the cerebral cortex.
- Alteration of consciousness occurs and period of confusion follows the event.
- Lasts 1-3 minutes and may consist of automatisms (lip smacking, chewing, swallowing, grimacing) or stereotypies- formerly complex partial seizures.
- Tx: AEDs/ EEG/ MRI with seizure protocol.

Case 17

- A 50-year-old woman was referred for evaluation of a movement disorder.
- When asked about her troublesome son she lies down, turns her head to the left and begins to shake violently.
- This continues for 5 full minutes. When it's over, she gets up and drives home.

Non-epileptic seizures (dissociative)

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- Characterized by sudden and time-limited disturbances of motor, sensory, autonomic, cognitive, and/or emotional functions that can mimic epileptic seizures.
- These events are **not** caused by abnormal brain activity (physical or mental stress).
- May occur in a patient who also has seizures.
- Treatment is psychotherapy.

Case 18

- A 56-year-old woman and many members of her family do not recognize other people's faces.
- She only recognizes people once she hears their voice.
- She was born with an intact visual cortex and did not have any brain damage.

Congenital Prosopagnosia

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- Inability to recognize faces.
- Cannot recognize close friends and relatives or pictures of famous people.
- Runs in families and does not involve a brain lesion.
- Prosopagnosia may occur as an acquired condition (due to bilateral lesions of the temporo-occipital area) or as a hereditary condition.

Case 19

- A child complained that everything in the room felt very small.
- The mother explained that the child recently had a viral infection.



Alice in Wonderland Syndrome

- A perceptual disorder involving brief, transient episodes of visual distortions.
- Can occur in conjunction with viral infections (EBV most common)
- Objects or body parts are perceived as altered in various ways.
- No permanent brain abnormalities are seen.

AIWS: Causes

- Other infectious causes (varicella, H1N1, coxsackievirus, typhoid fever, scarlet fever)
- Migraine
- Psychiatric disorders (major depression, schizophrenia)
- Epileptic seizures
- Medications (Topiramate, Risperidone, Montelukast, others)
- CNS lesions: infarct, tumor, hemorrhage
- Substance induced (cocaine, cannabis, MDMA, LSD others)

Case 20

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- A patient has an amputation. Following the amputation he swears that he has pain where his leg was.



Phantom Limb Syndrome

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- Painful sensations referred to the missing limb (could also be breast, rectum)
- This condition was first described in the 16th century.
- Wars and land mines-responsible for many of the traumatic amputations over the years.
- Females and upper extremity amputation are at higher risk.
- Mirror therapy is used with a variety of other options.



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Summary Slide

- Distinguish neurologic disorders from psychiatric disorders.
- Identify when a patient is causing harm to him/herself or someone is harming them.

Core References:

- [Uptodate.com](https://www.uptodate.com)
- [Ncbi.nlm.nih.gov](https://ncbi.nlm.nih.gov)

Lecture and Lab Feedback Form:

<https://comresearchdata.nyit.edu/redcap/surveys/?s=HRCY448FWYXREL4R>